

Tenecteplase (TNKase) Quick-Use Sheet – Acute Ischemic Stroke

Dosing Chart (IV Bolus Over 5 Seconds)

Weight (kg)	Dose (mg)	Volume (mL at 5 mg/mL)
< 60	15	3.0
60–70	17.5	3.5
70–80	20	4.0
80–90	22.5	4.5
≥ 90	25	5.0

Max dose in Acute Ischemic Stroke: 25 mg (5 mL)
Administer as single IV bolus over 5 seconds

Vial Sizes, Reconstitution & Wastage

Vial Size	Diluent Volume	Final Conc.	Max Dose Drawn	Wastage
25 mg	5 mL SWFI	5 mg/mL	5 mL (25 mg)	Dose dependent
50 mg	10 mL SWFI	5 mg/mL	5 mL (25 mg)	At least 5 mL (25 mg), then dose dependent

Wastage Key Points:

- 25 mg vial: Minimally wasted, preferred for AIS use
- 50 mg vial: Significant wastage (at least 25 mg for **all AIS patients**)
- Do not pool excess drug from multiple vials
- Document discarded volume per institutional policy

Preparation & Administration

Reconstitution:

- Use only accompanying SWFI included in package (5 mL for 25 mg; 10 mL for 50 mg)
- Swirl gently, do not shake
- Final concentration: 5 mg/mL

Administration:

- Draw up appropriate dose based on patient weight
- Push IV bolus over 5 seconds
- Flush IV line with NS only before and after administration (avoid D5W-incompatible)

Because TNKase contains no antibacterial preservatives, reconstitute immediately before use. If not used immediately, refrigerate solution at 2°C to 8°C (35°F to 46°F) and use within 8 hours. DO NOT FREEZE. Final concentration of TNKase is 5 mg/mL.

Onset, Duration, and Half-Life

Onset: Within minutes

Half-life: Biphasic

- Initial ~20 min
- Terminal ~90–130 min

Duration of action: ~2–4 hours

Monitoring:

- Neuro status q15 min for first 2 hours
- BP management & bleeding precautions
- No anticoagulants/antiplatelets for 24h

References:

<https://www.tnkase.com/content/dam/gene/tnkase/pdfs/tnkase-ais-dosing-card.pdf>

<https://www.tnkase.com/stemi.html>

https://www.gene.com/download/pdf/tnkase_prescribing.pdf